

NOTICE OF FUNDING OPPORTUNITY (NOFO)

PFQHC-2026-Q2A

Proactive Health Screening Incentive Program for Non-Mydriatic Automated Diabetic Retinopathy Screening and Monitoring

Application Due Date:

July 31, 2026, 11:59 pm Pacific Standard Time

Issuance Date: June 1, 2026

Program Application

This is a one-step application process

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WHO WE ARE

Proactive FQHC is a dedicated 501(c)(3) non-profit organization that advocates for and supports Community Health Centers nationwide that are Federally Qualified Health Centers (FQHC), FQHC Look-Alikes, or Native American Health Centers.

Our mission is to enhance community health through proactive health screening & monitoring, ensuring early detection and better patient outcomes.

Our primary objectives are advocating to Congressional legislators and other funding sources for financial incentives that encourage Community Health Centers to improve proactive health screening and monitoring. These incentives support Community Health Centers in expanding proactive patient screenings. The current programmatic focus area is retinopathy, a complication of underlying medical conditions like type 1 or type 2 diabetes, or other systemic and ocular causes frequently affecting underserved populations.

Read more at: [About - Proactive FQHC Inc](#)

GRANT SUMMARY

Funding Opportunity Title:	Non-Mydriatic, Automated Diabetic Retinopathy Screening and Monitoring
Funding Opportunity Number:	PFQHC-2026-Q1A
Due Date for Applications:	July 31, 2026, 11:59 pm Pacific Standard Time
Purpose:	To provide eligible Community Health Centers with a grant to support a three-month trial rental of one enhanced non-mydriatic, automated diabetic retinopathy screening camera, accelerating access to high- quality retinopathy screening in the communities they serve
Program Objective(s):	a) Encourage Community Health Centers to adopt the use of enhanced non-mydriatic, automated diabetic retinopathy screening technology in the routine primary care setting workflow to quickly screen diabetic or hypertensive patients and identify patients with findings indicative of retinopathy for additional follow-up with a specialist. b) Protect the vision of underserved populations by measurably increasing the percentage of eligible diabetic and hypertension patients screened or monitored c) Provide services regardless of the patient's ability to pay as determined by the organization's current practice. d) Collect patient service statistics from the Community Health Center, or its technology provider to report screening accomplishments. e) Within one year, document a significant increase in the health system's number eligible diabetic and hypertension population screened for retinopathy. f) Increase the proportion of retinopathy identified at earlier (mild/moderate) stages, relative to advanced disease at the time of screening, including by screening younger eligible patients
Period of Performance:	June 1, 2026 through June 1, 2027 Award funds may be used to support a minimum of three (3) months of eligible project costs within this period. Awardees are required to implement and actively use the automated diabetic retinopathy screening camera for a minimum of three (3) consecutive months during the period of performance. The three-month utilization period shall begin upon completion of camera installation, staff training and related clinic workflow adjustments. Award funds may be used to support eligible project costs incurred during this required three-month utilization period. Continued use beyond three months is encouraged but not required under this NOFO.

Eligible Applicants:	Your organization must be a Federally Qualified Health Center (FQHC), FQHC Look-Alike, or Native American Health Center a) Provide your organization's currently valid Internal Revenue Service (IRS) EIN or a statement from a state official certifying your organization has a nonprofit status b) Your organization will provide these services regardless of patients' ability to pay, consistent with your current policies and practices c) Applicants must comply with all applicable federal, state, and local laws and regulations
Anticipated Available Funding and Estimated Number of Awards:	Approximately \$24,000 Up to five (5) awards, one per eligible organization, of up to \$4,500 per award will be made Proactive FQHC reserves the right, prior to issuance of any Notice of Award, to modify the number of awards, individual award amounts, and/or total funding available under this NOFO, including in response to additional matching funds, changes in available resources, or programmatic considerations.
Cost Responsibility	Applicants are responsible for any project-related costs not covered by this grant award. There is no fixed minimum cost-share requirement. Applicants must demonstrate the capacity to support and sustain project activities for the duration of the award period, including covering any additional or unforeseen costs necessary to implement the proposed screening services. a) Each award provides funds for you to pay toward up to 3 months of the retinopathy camera rental fee for one camera plus a travel case if required for your usage profile. b) Awardees are responsible for any per-patient screening fees not covered by the awarded grant funding charged by the camera service provider, if applicable.
Other Requirements	a) When you submit your application, you certify that in your organization that you and your principals (e.g., program director) are authorized to participate in receiving award funds to carry out this proposed project b) If awarded a grant, you will need to provide wire transfer information to receive the funds at your bank. You must confirm receipt of funds to the Proactive FQHC Contact email address c) Your subrecipients and subcontractors must comply with the award requirements (including public policy requirements) that apply to you d) Write your application in English and use U.S. dollars where appropriate

Data Collection and Reporting Requirements	Participating health centers (or their technology providers) will submit de-identified patientlevel data on the 1st and 15th of each month, including
Data: Service Utilization Metrics:	a) Total number of patients screened b) Service location (for geographic trend analysis) c) Screening dates
Data: Patient demographics (de-identified):	d) Age group (by decade) e) Race/ethnicity f) Gender (as reported) g) Native language h) Primary diagnosis (Diabetes type 1 or 2, hypertension, both)
Data: Clinical Screening Data	i) Retinopathy screening history (first-time screening vs. established monitoring) j) Screening result (no findings indicative of retinopathy; findings indicative of retinopathy, where available: mild, moderate, severe, proliferative) k) Change in retinopathy status from previous screening (if applicable: new/improvement/stable/progression) l) Referral to ophthalmology (yes/no; if yes, urgency level) m) Referral completion status (to be tracked at subsequent data submissions)
Data: Primary Outcome Measure:	Within 6 months of camera installation, demonstrate an increase in the proportion of the health center's eligible diabetic and hypertensive patient population screened for retinopathy, relative to a baseline established using the 6-month period prior to implementation
Data: Secondary Outcome Measures:	• Increase in the proportion of retinopathy findings identified at early or moderate stages • Reduction in time from diabetes or hypertension diagnosis to first retinopathy screening • Ophthalmology referral completion rates among patients identified with actionable screening findings
Proactive FQHC Contacts:	Administrative, fiscal, or program questions: Richard Parenteau Hilal Aouam Call: 650-260-3358 Email: grants@proactivefqhc.org

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Non-Mydriatic Automated Diabetic Retinopathy
Screening and Monitoring**

Organization Name:

Organization Address:

Date: _____

Submitting Officer Name:

Submitting Officer Signature

Email:

Telephone + ext:
